



PERFORMANCEOVERVIEW

Medical Report

Physiotherapy & Rehabilitation Platform

Chronic Lumbar Disc Herniation with Sciatica

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #22

Clinical Details

Doctor: Dr. Alarkany

Date: 2026-04-28

Description: Patient presents with persistent radiating pain from the lower back down to the right calf. Diagnosed with L4-L5 disc protrusion. Pain is aggravated by prolonged sitting and forward bending.

Physical Measurements

Height: 178 cm

Weight: 85 kg

Blood Group: A+

Health History

Smoker: No

Chronic Diseases: No

Activity Level: Moderate

Smart Clinical Analysis

Clinical Explanation:

The patient presents with persistent radiating pain from the lower back to the right calf, attributed to an L4-L5 disc protrusion. This condition is characterized by nerve root irritation, leading to radicular symptoms. The patient's pain exacerbates with prolonged sitting and forward bending, indicating a need for careful management of activities that may increase intradiscal pressure. The rehabilitation strategy will focus on pain relief, improving functional mobility, and strengthening the core and lower extremities while ensuring that the patient avoids aggravating movements.

Immediate Physiotherapy Recommendations:

A tailored physiotherapy program should be initiated, focusing on gentle range of motion exercises, core stabilization, and lower extremity strengthening. Modalities such as heat or electrical stimulation may be beneficial for pain management. Education on posture and body mechanics during daily activities is crucial to prevent exacerbation of symptoms.

Precautions:

The patient should avoid prolonged sitting, forward bending, and heavy lifting. Activities that involve twisting or high-impact movements should also be limited to prevent further irritation of the lumbar spine.

Estimated Recovery Timeline:

Given the nature of the condition and the patient's progress, a recovery timeline of approximately 6 to 12 weeks is anticipated, contingent upon adherence to the rehabilitation program and response to treatment.

Risk Assessment:

Potential risks include worsening of symptoms, development of chronic pain, or progression of neurological deficits if the condition is not managed appropriately. Regular monitoring of symptoms is essential.

Red Flags:

Emergency warning signs include sudden worsening of pain, loss of bowel or bladder control, significant weakness in the lower extremities, or the presence of saddle anesthesia. Immediate medical evaluation is warranted if these symptoms occur.

Test Group: Range of Motion (ROM) Assessment

Test: Knee Flexion (Right)

Clinical AI Analysis

Normal Reference:
Normal knee flexion range is typically between 135 to 150 degrees for adults.

Clinical Interpretation:
The recorded knee flexion measurements show a progressive increase from 95 degrees to 115 degrees over the assessment period. This indicates an improvement in the range of motion, suggesting a positive response to rehabilitation efforts.

Progress Evaluation:
The trend demonstrates a consistent improvement in knee flexion, indicating enhanced mobility and potentially reduced pain levels during movement.

Physiotherapy Focus:
Continue to emphasize knee flexion exercises, incorporating both active and passive range of motion activities. Strengthening exercises for the quadriceps and hamstrings should also be included to support overall knee function.

LATEST RESULT

115.0

2026-04-28

HIGHEST

115.0

LOWEST

95.0

AVERAGE

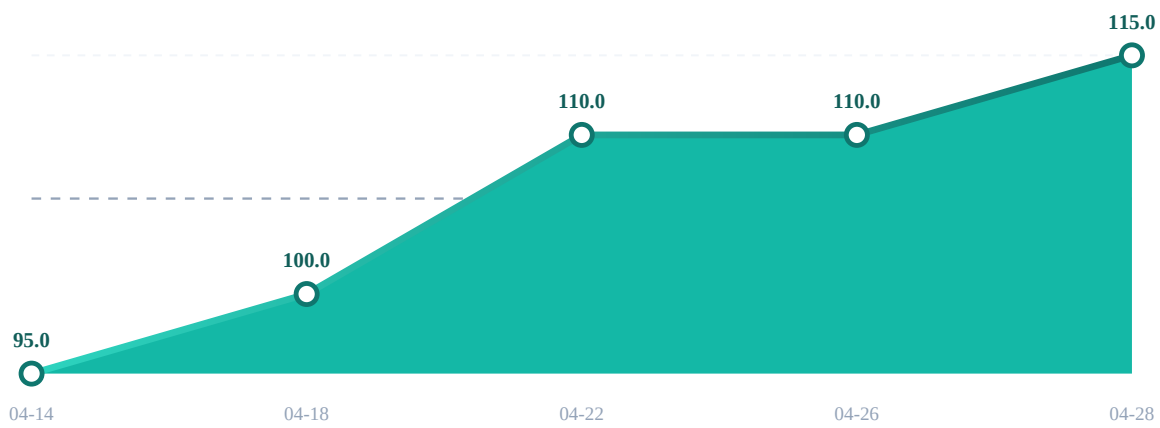
106.0

SESSIONS

5

OVERALL PROGRESS

Increased by 21.1% ↑ Since first test (2026-04-14)



DATE

RESULT

2026-04-14

95

2026-04-18

100

2026-04-22

110

2026-04-26

110

2026-04-28

115

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